

INDIVIDUAL CASE SAFETY REPORT (ICSR)

Prepared by: Dhruv Gupta | B.Pharm Student

1. Case Narrative (Fictional Training Scenario)

A 45-year-old woman was prescribed amoxicillin for a presumed bacterial respiratory infection. On the third day after starting treatment, she developed a generalized itchy skin rash. The medicine was discontinued after clinical review, and the rash resolved over the next several days. No rechallenge was performed. Exact dates, dose, concomitant medicines, allergy history, and laboratory findings are not provided.

2. Patient and Medical Information

Field	Training case information
Patient initials	A.B. (fictional; anonymized)
Age / sex	45 years / Female
Weight	Not reported
Indication	Presumed bacterial respiratory infection
Medical history / allergies	Not reported
Concomitant medicines	Not reported
Source	Educational case constructed for practice

3. Suspected Product Information

Field	Information
Suspected medicine	Amoxicillin
Drug class	Aminopenicillin antibiotic
Dose / route / frequency	Not reported
Therapy start	Day 1 (relative timeline)
Reaction onset	Day 3 after starting therapy
Action / withdrawal	Discontinued after clinical review
Batch / expiry	Not available
Rechallenge	Not performed

4. Adverse Reaction Details

Field	Assessment
Reaction	Generalized itchy skin rash
Onset	Approximately 3 days after initiation
Dechallenge	Positive: rash resolved after withdrawal, per scenario
Outcome	Recovered
Reaction treatment	Not specified
Coding	Exact MedDRA term not assigned in this educational report

5. Causality Assessment

Criterion	Evaluation
Temporal relationship	Supports association: reaction followed exposure.
Dechallenge	Supports association: improvement followed withdrawal.
Alternative causes	Insufficient information; underlying illness or other exposures cannot be excluded.
Rechallenge	Not reported; no re-exposure.
Overall training assessment	Possible to Probable/Likely range; provisional only, not a definitive formal assessment.

Rationale: Timing and improvement after discontinuation support a suspected relationship. Missing clinical details and incomplete assessment of alternative causes limit certainty. Formal WHO-UMC assessment requires fuller case information.

6. Seriousness, Severity, and Expectedness

Dimension	Training classification	Reason
Regulatory seriousness	Non-serious in this scenario	No death, life-threatening event, hospitalization, persistent disability, congenital anomaly, or other medically

		important outcome is described.
Intensity / severity	Not graded	The scenario describes generalized itch/rash but gives no clinical intensity scale or examination findings.
Expectedness	Undetermined	Must be checked against applicable amoxicillin reference safety information; do not infer from drug class alone.

7. ICSR Minimum Data Review

Minimum element	Status
Identifiable patient	Fictional initials and age/sex for practice
Identifiable reporter	Student author; no real clinical reporter
Suspected product	Amoxicillin identified
Suspected reaction	Skin rash/pruritus identified
Dates, dose, clinical details	Partially missing; follow-up required for a real case

8. Follow-up and Recommended Actions

For a real report, obtain exact treatment and event dates, dose, route, indication, concomitant medicines, allergy history, rash description, mucosal/systemic symptoms, treatment, recovery date, and source documents. Verify expectedness against current reference safety information. Do not deliberately rechallenge a patient merely to confirm causality.

9. Final Safety Summary

This fictional case describes a suspected amoxicillin-associated generalized itchy rash beginning on treatment Day 3 and resolving after withdrawal. Timing and dechallenge support a possible drug relationship. It is classified as non-serious based on the stated outcome; expectedness remains undetermined pending reference product information. Missing data and limitations are documented.

10. References

1. WHO–Uppsala Monitoring Centre. WHO-UMC system for standardised case causality assessment. <https://www.who-umc.org/>
2. ICH E2A: Clinical Safety Data Management—Definitions and Standards for Expedited Reporting.
3. ICH E2B(R3): Individual Case Safety Report (ICSR) electronic transmission specification.